

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>
To determine the most appropriate next step in treatment , we should consider the child 's symptoms and developmental history . The child is exhibiting symptoms of attention deficit hyper activity disorder (AD HD), such as difficulties with concentration , impuls ivity (interrupting others , hitting others), and rest lessness (frequent movement , expulsion from preschool). These symptoms have been persistent and are causing problems in various settings . Given the age of the child , the first line of treatment would likely be non-ph armac ological and non-in vasive .
</think>< answer> A . Behavior therapy </answer>

